

Orthopedic Consultation Note

Patient	Chris Johnson	Physician	Dr. James Reilly
Date of Birth	1981-11-12	Side	Right shoulder (dominant)
Occupation	FDNY Lieutenant, Ladder Company 24	Diagnosis	Partial-thickness supraspinatus tear
Referring	Dr. Sean Flanagan	ICD-10	M75.101

History of Present Illness

Patient is a 44-year-old right-hand-dominant career firefighter with 22 years of service presenting with chronic right shoulder pain and weakness with overhead activity. Symptoms have progressed over roughly three years, aggravated by forcible-entry work on the fireground, swinging a hurley while coaching, and sleeping on the right side. He reports intermittent night pain and catching with abduction above 90 degrees, managed conservatively with activity modification, ice, ibuprofen 400 mg as needed, and a home resistance-band program.

Past Medical & Surgical History

Sleep apnea	On CPAP, compliant	PTSD	In active therapy
Hyperlipidemia	On rosuvastatin	Prior surgeries	None
Tinnitus	Occupational, bilateral	Allergies	No known drug allergies

Examination & Imaging

Test / measure	Right shoulder	Interpretation
Active forward flexion	150 degrees	Mild loss
Abduction	135 degrees, painful arc	Impingement
External rotation strength	4 / 5	Cuff weakness
Empty-can (Jobe) test	Positive	Supraspinatus involvement
Hawkins-Kennedy	Positive	Subacromial impingement
MRI 2026-01-28	Partial supraspinatus tear	No full-thickness retraction

Assessment & Plan

Partial-thickness supraspinatus tear with impingement, symptomatic and functionally limiting for the patient's occupation. Operative and non-operative pathways were reviewed in detail.

- Operative:** arthroscopic rotator cuff repair with subacromial decompression. Sling four to six weeks, graduated return over roughly three months on restricted or desk duty with no fireground overtime.
- Non-operative:** continue supervised physical therapy twice weekly, corticosteroid injection if symptoms flare, re-image in six to twelve months. Lower short-term disruption, ongoing symptoms likely.

Decision pending Patient elected to defer surgery pending review of the financial impact of three months of lost overtime. Follow-up consult scheduled 2026-11-05 to finalize the decision. PT continues with Mike Chen.

Return-to-Duty Considerations

A firefighter's fitness-for-full-duty standard requires unrestricted overhead strength and grip. Post-operative clearance would follow an FDNY Bureau of Health Services evaluation after the twelve-week rehabilitation milestone. Electronically signed by Dr. James Reilly, Hospital for Special Surgery on 2026-02-05.

Current Medications

Medication	Dose	Purpose	Prescriber
Rosuvastatin	10 mg nightly	Cholesterol	Dr. Sean Flanagan
Ibuprofen	400 mg as needed	Shoulder pain	Self / PCP
No daily psychiatric medication	—	PTSD in therapy	Dr. Rachel Kovacs

Patient Instructions

- Continue the home resistance-band program and twice-weekly physical therapy.
- Ice after fireground activity or coaching; avoid repetitive overhead loading.
- Weigh the operative decision against the financial impact of three months of restricted duty.
- Return for the November 5 consult to finalize the surgical decision.

Follow-Up & Care Coordination

A copy of this note and the MRI report has been sent to the referring primary-care physician and to the treating physical therapist so the conservative program and the operative option stay coordinated. Should the patient elect surgery at the November consult, scheduling, pre-operative clearance through cardiology and the sleep service for the CPAP, and an FDNY Bureau of Health Services notification would follow. The patient was given written home-care instructions and the shoulder-service phone line for questions before the next visit.